

Revenue Projection: AI Ticket Intake

Product: AI Ticket Intake

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MSPbots' actual credit pricing and tier ARR are internal numbers I don't have access to. The figures here are directional. The logic matters more than the exact amounts. This section covers three things: how AI Ticket Intake creates value, how that value turns into revenue, and what the 12-month trajectory looks like across three scenarios.

1. Pricing Assumptions

There are four ways to think about pricing this feature. They're not mutually exclusive. Which one applies depends on the phase.

A. How much engineer time does AI Ticket Intake save

Compare tickets submitted through the normal system vs. tickets submitted through AI Ticket. Look at how much the time spent handling open tickets improves. Use the per-ticket cost to work out what AI Ticket is actually worth.

Right now, coordinators spend 3–10 minutes per ticket chasing missing context. After AI Intake, the target is under 1 minute. That's a success gate in GTM Phase 2. Each ticket saves 5–9 minutes.

At a conservative \$35/hr for coordinator time:

- 5 minutes saved $\times$ \$35/hr = ~\$2.90 recovered per ticket
- A mid-size MSP running 800 tickets/month = ~\$2,320/month in recovered labor value

This isn't what to charge. It's what to use to show the value. If MSPbots captures 10–15% of that, a \$200–\$350/month price point per MSP is reasonable, or the equivalent in credits.

B. How much time does a customer spend submitting a ticket without AI

Calculate how long customers spend filling out tickets in the system without AI assistance. That gives you the baseline. Then calculate how much time AI Intake saves in the submission process.

A non-technical user without AI guidance spends 8–12 minutes trying to fill out a ticket, guessing at fields, getting stuck, sometimes giving up. AI Intake guided mode brings that down to 3–5 minutes.

For small MSPs without a dedicated dispatcher, this number matters more than A. AI is doing the job a dispatcher would do. That's not just a time saving argument. It's a headcount argument.

This reinforces the secondary segment logic in the GTM: small MSPs (1–10 staff) don't have a dispatcher. AI Intake fills that role. The value per ticket is higher for them even at lower volume, so their willingness to pay is higher too.

C. Bundle into existing plans, no direct new revenue, just added value

AI Intake is included in existing subscription tiers. No new SKU, no per-ticket fee, nothing extra for customers to evaluate.

This is the right call for GA launch. The biggest risk at GA isn't underpricing. It's low adoption. If MSPs have to make a separate purchase decision, some won't bother. Bundled means zero friction. They just turn it on.

Revenue impact is indirect but real:

- **Retention:** MSPs using AI Intake are more embedded in the product. Harder to leave.
- **Tier upgrades:** AI Intake increases credit consumption. MSPs near their limit get an upsell prompt.
- **New logos:** For small MSPs, "included in your plan" removes the main objection. Get them in on AI Intake, expand to Triage later.

D. Charge based on the number of AI tickets submitted

Already in the GTM plan: 1 credit per AI Intake action, same pool as AI Triage. No new billing infrastructure needed. MSPs already know how credits work.

For a mid-size MSP running 800 tickets/month at 70% AI Intake adoption:

- 560 AI Intake actions/month
- At \$0.20/credit (MSPbots published add-on rate): \$112/month in incremental credit usage
- Main effect: MSPs on lower tiers hit their limit faster, which triggers an upgrade conversation

One risk: if MSPs feel like they're being charged for every action, some will turn off AI Intake to save credits. That defeats the purpose. This model works best after adoption is established and the value is proven, not as a barrier to getting started.

Recommended model by phase

Phase	Pricing approach	Revenue type
Beta (Month 1–2)	Free, no credit consumption	None; data collection only
GA mature (Month 7–12)	Bundled base + credit overage for heavy users	Incremental credit revenue + tier upgrades

2. Adoption Assumptions

Based on the GTM rollout timeline.

Starting point:

- Beta is free, so early sign-on is expected to be high. Real usage data starts in Phase 2.
- Post-GA, small MSP new logos: estimated 2–4/month, driven by AI Intake as the entry product.
- Activation rate = % of existing AI Triage users who enable AI Intake within 12 months.

Month	Conservative (30% activation)	Base (50%)	Optimistic (70%)
1–2 (Beta)	10 MSPs	10 MSPs	10 MSPs
4–6	40–50	60–80	90–110
10–12	70	110	160

The conservative scenario assumes limited in-product promotion and no Suite bundle. Base assumes normal GA execution. Optimistic assumes a strong beta case study, active in-product messaging (the BI dashboard trigger from the GTM plan), and Suite bundle launch in Month 9.

3. 12-Month Revenue Impact

Three revenue streams across three scenarios.

Stream 1: Credit tier upgrades

MSPs using AI Intake consume more credits. Those near their tier limit get an upsell prompt. Estimated 15–25% of active AI Intake users upgrade within 12 months.

Stream 2: AI Ticket Suite bundle premium

If launched in Month 9–12: Triage + Intake as a combined SKU, priced above Triage-only. New customers default to the Suite. Existing Triage-only customers migrate over time.

Stream 3: Net new customer acquisition

Small MSPs entering through AI Intake. Lower initial ARR per customer, but a clear expansion path into Triage and QA products.

12-month directional projection

Scenario	Active MSPs at Month 12	Primary revenue driver	Revenue shape
Conservative	~70	Credit overages only	Modest incremental ARR; revenue back-loaded
Optimistic	~160	Suite bundle + new logo acceleration	Material new ARR; AI Intake becomes a growth driver

Exact dollar amounts depend on tier pricing and credit costs, which are internal numbers. Revenue is mostly back-half loaded. Months 1–6 are investment. Months 7–12 are where tier upgrades and Suite upsell start to build.

4. Why This Revenue Logic Holds

AI Intake makes what MSPs are already paying for work better.

From the GTM: "Tickets with complete information are triaged 40% more accurately." AI Intake isn't adding a new feature. It's making the existing AI Triage product perform better. The whole product becomes more valuable, which makes customers less likely to leave.

Small MSPs get a dispatcher they don't have to hire.

For a 5-person MSP, replacing a part-time dispatcher with AI Intake costs a fraction of what hiring someone would. The PRD is explicit about this: non-technical users and small MSPs without coordinators are the core segment where AI Intake fills a missing role. It's not a \$56/month feature pitch. It's an operational cost argument.

The credit model aligns incentives.

When AI Intake performs well, MSPs use it more. When they use it more, credit consumption goes up. When credit consumption goes up, upgrading tiers happens naturally. The better the product works, the more natural the upsell becomes. What's good for the user and what's good for the business point in the same direction.

This isn't a standalone new revenue source. It's retention and expansion built on top of AI Triage. AI Intake makes existing customers harder to lose and gives customers who weren't ready to start with Triage a lower-friction way in.